CHAROTAR UNIVERSITY OF SCIENCE AND TECHNOLOGY

University Examination

Date: 07/06/2018 Day: Thursday Time: 10:00 AM to 10:30 AM Total Marks: 20

Student's ID No.	Answer Sheet for Multiple Choice Questions (MCQs),True or false and Match the following
	Examination : MPT- June 2018
Student's Name: ______________________________	Semester: 2nd
Student's Sign : ______________________________	Section : I
Supervisor's Name : ______________________________	Course Code : PT792
Supervisor's Sign: ______________________________	Course Name: Sports Traumatology

Important Instructions:

- There are two sections in the paper. **SECTION –I** contains 20 Questions and **SECTION – II** contains 3 Questions.
- **SECTION –I** should be answered in the question paper itself & **SECTION – II** should be answered in separate answer book.
- Marks on the right side suggest the total marks of that question.
- Maximum time allotted for **SECTION – I** is 30 minutes Question paper for **SECTION – II** will be given once the **SECTION – I** is completed within the stipulated time and returned. However, Candidates who complete **SECTION – I** earlier than 30 minutes can collect main question paper for **SECTION –II** immediately after handing over the **SECTION-I** question paper with answers.
- **SECTION – I** should be attached with the main answer book at the end of the examination and should be submitted to the supervisor.

SECTION-I

Multiple Choice Questions: (MCQs) **(10 × 1 = 10)**

(Encircle the correct answer)

1. Consider the following statements about carpal tunnel syndrome:

1. It may occur in acromegaly
2. It is common during pregnancy
3. It causes delayed nerve conduction
4. It is associated with wasting of abductor pollicis longus

a. 1, 2 and 3 are correct
b. 2, 3 and 4 are correct
c. 1, 2 and 4 are correct
d. 1,3 and 4 are correct

(P.T.O)

2. All are true for Dupuyteren's contracture except:

- **a.** Usually 4th finger is involved
- **b.** Bilateral disease is rare
- **c.** Surgical release is useful
- **d.** May be associated with Pyronie disease

3. A 6-year old kid fell on right sided forearm region and develops fracture in dorsal surface of mid region of radius. The best treatment is:

- **a.** Antibiotics and sedative
- **b.** Bone plating and external fixation
- **c.** Slab with weight for bone remodeling
- **d.** Immobilization

4. Recurrent dislocation is most common injury seen in throwers. Which of the following is not an important cause for the same?

- **a.** Tear of anterior capsule of shoulder
- **b.** Associated fracture neck of humerus
- **c.** Tear of glenoid labrum
- **d.** Freedom of mobility at shoulder

5. Which of the following statement is **not** correct regarding fracture of scaphoid?

- **a.** It is the most commonly fractured carpal bone
- **b.** Persistent tenderness in the anatomical snuffbox is highly suggestive of fracture
- **c.** Immediate X-ray of hand may not reveal fracture line
- **d.** Mal union is a frequent complication

6. Fracture lateral condyle of humerus is a common injury in children. Which one of the following is the most ideal treatment for a displaced fracture lateral condyle of the humerus in a 7-year old child?

- **a.** Open reduction and plaster immobilization
- **b.** Closed reduction and plaster immobilization
- **c.** Open reduction and internal fixation
- **d.** Excision of fractured fragment

7. A 12-year old child presents with tingling sensation and numbness in the little finger and presents with history of fracture in elbow 4 years back. The probable fracture is:

- **a.** Lateral condyle fracture of humerus
- **b.** Injury to ulnar nerve
- **c.** Supracondylar fracture humerus
- **d.** Dislocation of elbow

8. Non-union is a very common complication of intracapsular fracture of neck of femur. Which of the following is not a very important cause for the same?

- **a.** Inadequate immobilization
- **b.** Inadequate blood supply
- **c.** Inhibitory effect of synovial fluid
- **d.** Stress at fracture side due to muscle spasm

9. A patient with hip in adduction and medial rotation is unable to move. The most probable diagnosis is:

- **a.** Posterior dislocation of head of femur
- **b.** Fracture shaft of femur
- **c.** Fracture neck of femur
- **d.** Sciatic nerve compression

(P.T.O.)

10. A 25year old football goalkeeper fell on the ground after being hit from behind and was unable to stand on right buttock region ecchymosis with external rotation and lateral border of foot touching the ground. The most probable diagnosis may be:

- **a.** Extracapsular fracture neck femur
- **b.** Anterior hip dislocation
- **c.** Intracapsualr fracture neck femur
- **d.** Posterior dislocation of hip

Identify the sentence as True or False: **(5 × 1 = 5)**
(Please tick mark the answer)

	True	False
11. March fracture seen in boxers usually involves fracture of 1st metatarsal.	☐	☐
12. Transverse fracture of medial malleolus involves rotation injury.	☐	☐
13. True Hangman's fracture typically involves odontoid fracture at C2.	☐	☐
14. Careless handling of suspected fracture cervical spine may result in intracranial hemorrhage with unconsciousness**.**	☐	☐
15. Fatigue fracture does not occur in metacarpal bones.	☐	☐

Match the following **(5 × 1 = 5)**

16.	Position for transfer of patient with fracture lumbar spine	**a.** Hyperflexion
17.	Injury to median nerve is best detected by	**b.** Neutral
18.	Ulnar paradox	**c.** Action of abductor pollicis
19.	Feature of Radial nerve injury at spiral groove	**d.** Loss of sensation of radial half of finger
20.	Cobb's angle	**e.** Intrinsic muscles
		f. Thumb, finger and wrist drop
		g. Kyphosis

(16 -), (17-), (18-), (19-), (20-)

CHAROTAR UNIVERSITY OF SCIENCE & TECHNOLOGY
2nd Semester of MPT Examination
June 2018
PT792 Sports Traumatology

Date: 06/06/2018 **Time: 10:30 AM-01:00 PM** **Maximum Marks: 50**

Instructions:

1. There are two Sections in the paper. **SECTION –I** contains 20 Questions and **SECTION – II** contains 3 Questions.
2. **SECTION –I** should be answered in the question paper itself & **SECTION – II** should be answered in separate answer book.
3. Marks on the right side suggest the total marks of that question.
4. Maximum time allotted for **SECTION – II** is 2 Hr 30 min.
5. Draw the figure where necessary.

SECTION-II

Q. 1. Short answers (5 out of 7) (Answer not exceeding 50 words) **(5 × 2 = 10)**

- **a.** Any 2 causes of patellar tendinopathy
- **b.** Signs and symptoms for Keinbock disease
- **c.** Handling & Transferring cervical injury player
- **d.** Signs and symptoms of Concussion
- **e.** Any 2 risk factors for IT Band syndrome
- **f.** Mechanism of injury in Mallet finger
- **g.** Differentiate between Tendinitis & tendinosis

Q. 2. Short Notes (4 out of 5) (Answer not exceeding 150 words) **(4 × 5 = 20)**

- **a.** Primary & Secondary emergency assessment
- **b.** On field heat stroke management
- **c.** Components of Pre-participation screening and evaluation for injury surveillance in football
- **d.** Risk factors, causes and preventive strategies for stress fracture tibia in runners
- **e.** Injury patterns in contact sports

Q. 3. Essay (2 out of 3) **(2 × 10 = 20)**

- **a.** Write in detail about fracture management in post surgical phase involving basketball player with fracture neck femur managed with ORIF.
- **b.** A 25 year old discus thrower has had pain running down the elbow and wrist from the dominant right shoulder while executing the final stages of movement. Complaint is extremely uncomfortable with progressive loss of strength in the last 3 months. Discuss in detail about the assessment plan with possible intervention strategies and differential diagnosis.
- **c.** Discuss in detail about managing a collapsed athlete during a game of football and safe transfer strategies to prevent any serious damage to cervical spine.